

Editor/Author Correspondence

Editor
2026-04-17
03:36 PM

Subject: [JMI ms#96541] Editorial Decision/Comments on "Healthcare Analytics Challenges: A Three-Pillar Framework Connecting Analytics Maturity, Workforce Agility, and Technical Enablement"

Delete

Decision E2: (not accepted for JMIR Medical Informatics)

Dear Samuel Harrold,

Thank you very much for your submission to JMIR Medical Informatics. The handling Editor has completed review of the article. At this time, the manuscript cannot be considered for publication in JMIR Medical Informatics.

Although this seems to be an interesting manuscript, the handling Editor deemed the work not suitable for consideration in JMIR Medical Informatics because of one or more of the following reasons:

- (X) The manuscript may be publishable in principle but was rated as "low impact".
- (X) The manuscript seems to have methodological weaknesses/limitations.

We should also note that JMIR Medical Informatics is trying to position itself as a leading eHealth journal, so we have to be selective in what we publish. If we do not accept your manuscript then this is not necessarily an indication of the scientific quality of your work, but sometimes more a reflection of our editorial priorities and interests.

Transfer the manuscript to another JMIR Publications journal and re-instate

For more information about JMIR Publications transfer and reinstatement process, please read: <https://support.jmir.org/hc/en-us/articles/115001714547-How-do-I-request-a-manuscript-transfer-to-another-journal-if-my-paper-was-rejected-or-I-am-unable-to-address-reviewer-comments->

JMIR Publications offers "portable peer-review" (also called cascading peer-review), meaning that manuscripts and their peer review comments, if available, can be transferred to other journals by the same publisher without going through a new peer-review process (for manuscripts rejected after at least one peer-review round). For newly submitted manuscripts that have not undergone peer-review, a new submission process is not needed with reinstatement and transfer, however, peer review would still be required. See <https://jmir.zendesk.com/hc/en-us/articles/115002462188> for more information.

Please let us know if you wish to reinstate and transfer the manuscript by contacting ed-support@jmir.org within four (4) weeks of this notification.

Note that different JMIR Publications journals have different Article Processing Fees. Please check <https://jmir.org/fees/article-processing-fees> to verify the APF for the new journal.

Thank you very much again for choosing JMIR Medical Informatics and giving us the opportunity to review your interesting work. We realize that this may be disappointing news, but we hope that you will continue to consider JMIR Medical Informatics or another JMIR Publications journal as an outlet for your high-quality research work.

Best regards,

Anastasia Shelomentseva
anna.shelomentseva@jmir.org
Editorial Assistant

[Ms link for editor only: <https://medinform.jmir.org/editor/submissionReview/96541>
For section/guest editor:
<https://medinform.jmir.org/sectionEditor/submissionReview/96541>]

Questions? Check out our knowledge base at <https://support.jmir.org/hc/en-us> !
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JMIR Medical Informatics has a new Journal Impact Factor of 3.8 (Source: Journal Citation Reports 2025 from Clarivate); see <https://medinform.jmir.org/announcements/590>

JMIR Medical Informatics is indexed in PubMed, PubMed Central, DOAJ, SCOPUS, and SCIE (Clarivate)

JMIR Medical Informatics <http://medinform.jmir.org/>

Editor
2026-04-20
03:22 PM

Subject: [MxM ms#96541] Suggest Transfer for Your Recently Submitted Manuscript

[Delete](#)

Dear Samuel Harrold:

Thank you very much for your message.

After editorial review, your manuscript was deemed better suited for another JMIR Publications journal, namely one of the following journals:

- (X) Online Journal of Public Health Informatics
- (X) Interactive Journal of Medical Research (i-JMR)
- (X) JMIR Bioinformatics and Biotechnology **

**Please note that if you agree to transfer to JMIR Bioinformatics and Biotechnology and the Editor of the journal accepts the manuscript for publication after peer review, then JMIR Publications can offer a one-time APF waiver for the authors to use only for this journal and for this manuscript.

*Please check <https://jmir.org/fees/article-processing-fees> to verify the APF for the new journal.

Manuscript URL: <https://manuscripts.jmir.org/author/submission/96541>
Username: samuel.harrold@gmail.com

Please let us know within ONE (1) week whether or not you agree to a manuscript transfer and, if so, we will seek approval of the transfer from the Editor-in-Chief of the respective journal.

Important: If we do not hear from you within ONE (1) week regarding your preferences about the above options, we will release the manuscript back to you so you can decide on the best next steps for your work. The submission will be marked in our system as "Rejected with option to reinstate" to the suggested journal. Then, you may choose to reinstate your paper to one of the suggested journal(s).

Note that while we await your response about transfer, your manuscript is reassigned to the Transfer Hub.

For more information about JMIR Publications transfer process, please read: <https://support.jmir.org/hc/en-us/articles/115002462188-Why-has-my-article-been-transferred-to-another-journal-or-a-transfer-has-been-suggested-and-what-are-my-options>

Please let us know how you would like us to handle your submission by replying to this message or contacting ed-support@jmir.org.

Best regards,

Anastasia Shelomentseva
anna.shelomentseva@jmir.org
Editorial Assistant

[Ms link for editor only: <https://manuscripts.jmir.org/editor/submissionReview/96541>
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Editor
2026-04-23
04:21 PM

Subject: Healthcare Analytics Challenges: A Three-Pillar Framework Connecting Analytics Maturity, Workforce Agility, and Technical Enablement

Samuel Harrold
Wed, Apr 22, 10:57 PM (17 hours ago)
to me, JMIR

Dear Anastasia Shelomentseva,

Thank you for your prompt response and for the transfer options provided.

I would like to proceed with a transfer to the Interactive Journal of Medical Research (i-JMR).

Please let me know if any additional information is needed to complete the transfer.

Best regards,
Samuel Harrold
samuel.harrold@yuimedi.com
Yuimedi, Inc.

Editor
2026-06-05
02:17 PM

Subject: [IJMR ms#96541] Editorial Decision/Comments on "Healthcare Analytics Challenges: A Three-Pillar Framework Connecting Analytics Maturity, Workforce Agility, and Technical Enablement" [Delete](#)

Decision D: (major revision & re-review)

Dear Samuel Harrold,

Your manuscript (#96541) in Interactive Journal of Medical Research has now been reviewed. The relevant reviewer/editor comments are listed at the end of this email.

Interactive Journal of Medical Research cannot offer publication of the paper in its current form. Please address the issues mentioned below and submit your revised manuscript for review. Substantial revisions may be noted in the comments.

Please contact us as soon as possible if you are unable to fully address the identified issues, or if you are requesting transfer to another JMIR Publications journal. See <https://jmir.zendesk.com/hc/en-us/articles/115001714547>

The due date for your revision and responses to reviewers is 2026-07-03.

This deadline can be extended once (see <https://jmir.zendesk.com/hc/en-us/articles/115001333167>).

Revision & Resubmission Instructions

To upload a revised version of your manuscript, go to <https://www.i-jmr.org/author/submissionReview/96541> and do the following:

- 1) Revised Manuscript: Use the "Revised Ms Upload" file-upload function and upload a clean (non-tracked) revised Word document of your manuscript.
- 2) Author Response to Comments: Copy and paste point-by-point responses to reviewer/editor comments into the notification email to the editor. Please do not upload a separate file containing responses. There are no formatting options (e.g., bold, italics) in this field.
- 3) Upload a revised version with tracked changes (or highlighted sections of text where changes were made) as a supplementary file, in addition to providing a clean version under "Revised Ms". See <https://jmir.zendesk.com/hc/en-us/articles/115001400448> for details on how to respond to reviewer comments and upload a revised manuscript. Supplying a version of the manuscript with tracked changes facilitates the review process.

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Author names and metadata

All co-authors should verify how their names and affiliations will be displayed by informing the submitting author if edits are required.

Also note that for the final publication (if accepted), metadata (author names and affiliations) will be taken from the database information you entered when submitting this article, NOT the manuscript file.

Thank you.

Best regards,

Matthew Balcarras, PhD
matt.balcarras@jmir.org
(Editor)

Interactive Journal of Medical Research
JMIR Publications

For status updates, please first read the FAQ (<https://support.jmir.org/hc/en-us/articles/115001358188>).

For scientific content questions only, please use the "Contact Editor" function on your manuscript link (<https://support.jmir.org/hc/en-us/articles/115001332927>).

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[Ms link for editor only: <https://www.i-jmr.org/editor/submissionReview/96541>
For section/guest editor: <https://www.i-jmr.org/sectionEditor/submissionReview/96541>]

EDITORIAL COMMENTS (Decision: D/major revision)

Thank you for your submission. Please see the comments below and provide a point by point response.

* Editorial and content comments *

- The central argument of your viewpoint is interesting and relevant. However the manuscript could be stronger by using better examples and interacting more with relevant and recent literature.

* Formatting/structure comments *

- All in-text references must be numbers in square brackets. Do not use the author-year system, round brackets, or superscripts. Follow our instructions for authors for references: <https://support.jmir.org/hc/en-us/articles/115001333067-How-should-references-be-formatted-Which-journal-style-should-I-choose-when-using-Endnote->

- Do not number your headings or use italics or bold for emphasis in place of subheadings. Ensure each section has more than one subheading; otherwise, remove it. All subheadings must be under Introduction, Methods, Results, and Discussion.

- (required) Funding Statement - Add a funding statement, distinct from the Acknowledgements. If no funding was involved, please state: "The authors declared no financial support was received for this work." More information: <https://support.jmir.org/hc/en-us/articles/360015982471-How-should-the-Acknowledgments-section-be-formatted->

- (required) Disclose whether generative AI was used in any portion of the manuscript generation: if used, it is mandatory to include a written statement in the manuscript, even if you disclosed use at the time of submission (e.g., in cover letter or submission form). If none, authors should attest to this in the response to comments. <https://support.jmir.org/hc/en-us/articles/13387268671771-Do-you-allow-the-the-use-of-ChatGPT-or-other-generative-language-models-and-how-should-this-be-reported-> Authors may optionally use this tool to generate a complete disclosure: <https://panbibliotekar.github.io/gaidet-declaration/index.html>

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Note that it is the authors' responsibility to ensure adherence to JMIR Publications style and formatting, including final word count.

Addressing formatting issues and any remaining comments during the revisions period speeds up the production process in the event of acceptance.

Further Editorial / Peer-reviewer Comments:

External Peer-Review Report(s)

Reviewer Q:
General comments
=====

This paper addresses an important issue regarding the internalization of tacit healthcare analytics knowledge within healthcare organizations which usually leaves the organization due to high turnover rate among experts. Authors recommend a human in the loop mechanism to translate the queries of users into implementable SQL queries and run them through experts for validation and potential correction. The validated queries are then stored in a knowledge base for future references. While the concern regarding knowledge restoration is a valid challenge across all organizations, including healthcare, the proposed framework has certain major issues which require further attention.

Specific comments
=====

Major comments

1. The manuscript frequently claims low maturity level by referring to certifications such as HIMSS. It is worth noting that organizations primarily avoid such assessments due to high costs. Therefore, concerns about self selection bias requires attention. In addition, using such acronyms without proper explanation adds to complexity of the manuscript.
 2. The practicality and efficiency of the proposed framework depends on the volume, diversity and frequency of the queries generated by users. A deeper discussion around these aspects of the the query generation activities within a healthcare organization is needed. For instance, if an organization requires only 50 frequent queries, then the problem can be addressed via a proper documentation or even the use of an industry level knowledge base.
 3. One of the biggest concerns which requires further investigation is the whether the human in the loop can actually perform better than AI or LLMs. an empirical investigation of the performance of experts in detecting and correcting AI errors in this context can significantly add to the value of the manuscript. If that fails, the effectiveness of the model will be a subject of debate.
 4. Finally, if the data is constantly changing, as has been argued in the manuscript, then the tenure of the informatics experts might not be enough to address the maturity issue. in addition, trainings and awareness will be key players too.
- ...

Minor comments

5. please make sure to clearly explain the acronyms used throughout the manuscript, otherwise it will be complex to follow the flow of the paper.
6. some of the reference, specially those arguing about the tenure of healthcare IT workers, is more than 20 years old and should be replaced.

Please help us improve the quality of reviewers by providing a rating for each review using the URLs provided below (your ratings are anonymous and only seen by the editor):

TO RATE REVIEWER Q: CLICK ON ONE OF THE FOLLOWING URLS

1. very poor/misleading/wrong statements DO NOT USE REVIEWER AGAIN:

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3. below average (too short or various weaknesses):

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5. excellent / very useful review:

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Reviewer T:
General comments
=====

This viewpoint paper presents the author’s argument regarding persistent workforce turnover in the healthcare analytics domain and the associated challenges of retaining tacit knowledge. The author highlights that traditional apprenticeship models are increasingly ineffective, particularly given the short tenure of many professionals in healthcare settings. Although the issue itself is not novel, it remains a persistent and unresolved problem over time. To address this challenge, the author proposes leveraging artificial intelligence to capture and preserve tacit knowledge. The paper introduces a useful framework—human-in-the-loop knowledge governance (HITL-KG)—which aims to shift the locus of organizational knowledge from volatile human memory to durable semantic artifacts. These artifacts, termed “Validated Query Triples,” consist of a natural language intent, executable SQL, and accompanying rationale metadata, thereby enabling more sustainable knowledge retention and transfer.

Specific comments
=====

Major comments

Major Comments

1. Definition of Key Terms

Please clearly define the key terms used in the title—analytics maturity, workforce agility, and technical enablement. While their general meanings can be inferred, academic rigor requires that the author provide explicit definitions, ideally grounded in or adapted from existing literature. These are not novel constructs, and established definitions are likely available and should be cited to strengthen conceptual clarity.

2. Redundancy of Content

Some content appears repetitive. For example, statistics related to CIO tenure and turnover rates are introduced in the Introduction but are reiterated again in Section 2.1. Consider consolidating this information to avoid redundancy and improve the overall flow of the manuscript.

3. Clarification of Figure 1 (Framework Dynamics)

Figure 1 presents the architecture of the human-in-the-loop knowledge governance framework. However, the diagram currently depicts a static knowledge base and set of best practices. In reality, best practices are dynamic and evolve with new technologies and changing business rules. It would be more appropriate to reflect this iterative nature in the figure. Specifically, Step 7 (organizational memory) should feed back into Step 1 (knowledge base and best practices), following validation from prior steps, to illustrate continuous learning and system updating.

4. Use of Outdated Sources

Section 4.2 references a statistic from 2004. Given the rapidly evolving nature of healthcare analytics and workforce dynamics, this source is outdated. Please update with more recent evidence to ensure the argument reflects the current state of the field.

Minor Comment

• Please provide complete reference details for all citations, including volume, issue number, and page ranges. For example:
Ang, S., & Slaughter, S. (2004). Turnover of information technology professionals: The effects of internal labor market strategies. ACM SIGMIS Database: The DATABASE for Advances in Information Systems, [volume(issue)], pages.
<https://doi.org/10.1145/1017114.1017118>

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3. below average (too short or various weaknesses):

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4. average (solid review with minor weaknesses):

[https://www.i-jmr.org/rev/authorRating?
ms=96541&re=477063&ro=1&ra=4&au=ca9b670f90](https://www.i-jmr.org/rev/authorRating?ms=96541&re=477063&ro=1&ra=4&au=ca9b670f90)

5. excellent / very useful review:
[https://www.i-jmr.org/rev/authorRating?
ms=96541&re=477063&ro=1&ra=5&au=ca9b670f90](https://www.i-jmr.org/rev/authorRating?ms=96541&re=477063&ro=1&ra=5&au=ca9b670f90)

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